ANTENATAL CARE SCHEDULE CHEAT SHEET

This table was designed on local and national guidelines to support clinicians in scheduling antenatal care for some common conditions. This is not an exhaustive list. Please write reason if you are requesting extra appointments to what is recommended. Clinics and investigations can be declined if reasons are not included. If you have comments or feedback on this table, please fill in this form : Antenatal Scheduling Chart Suggestions and Improvements Box – Fill in form

We would appreciate your feedback on this schedule. Antenatal Scheduling Feedback – Fill in form

Advanced maternal age- > 40 ..... ***2***

Anaemia- ***2***

BMI 35-39.9 ..... ***2***

BMI>40 ..... ***2***

Chicken pox ***2***

Chronic Hypertension ***3***

Congenital uterine anomaly ***3***

Epilepsy ***3***

Fetal movements ***3***

REDUCED FETAL MOVEMENTS ***3***

FGR baby: EFW/AC <3rd centile or abnormal doppler ***4***

Fibroid uterus ***4***

Gestational diabetes ***4***

Herpes ***5***

Hyperthyroid ***5***

Hypothyroid ***6***

Inflammatory Bowel Disease ***6***

IVF pregnancy ***6***

Large baby - ***6***

Low BMI <18.5 ..... ***6***

Low PaPP-A (0.415 MoM) ***7***

Maternal antibodies ***7***

Placenta Praevia/ low lying at 20/40 ..... ***7***

Polyhydramnios ***7***

Previous Caesarean Section ***8***

Previous congenital abnormality ***8***

Previous PET ***8***

Routine care- low risk pregnancy ***9***

SGA baby - EFW/AC <10th centile ***9***

Sickle cell disease ***10***

Smoker / Vaping ***10***

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Twins – DCDA ***11***

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Type 1 and 2 Diabetes- prepregnant Diabetes ***12***

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Conditions and Guidelines	Antenatal Visits *schedule to follow routine care as minimum Midwifery visits should be considered when Obstetrician ANC are not required to promote shared care This list should be in conjunction with the Clinical Risk assessment guidelines and in support of the local guidelines	Examinations and tests	Ultrasound scan
Advanced maternal age- > 40	12 weeks: Aspirin risk assessment (add assessment) 16 weeks: Obstetrician ANC 32 & 36 weeks: Obstetrician ANC IOL – To offer from 39 weeks	28 weeks: one off SFH	4-weekly from 32 weeks till birth
Anaemia- **1st trimester < 110g/l** **2nd and 3rd trimester <105 g/l** **202511211517060.Anaemia and Ferinject Infusion in Pregnancy v3.1.pdf**	If anaemia start oral iron (unless known haemoglobinopathy then measure serum ferritin and start iron only if <30ug/l) - Ferinject if not tolerating orally - ANC referral if Hb<70g/l or if symptomatic -	Booking, 28- and 34-weeks bloods (Serum ferritin and hematinics if indicated. Check Haemoglobinopathies from booking)	Scans if Hb<80g/l (and every 4 weeks till birth)
BMI 35-39.9 **Raised Body Mass Index (BMI) in pregnancy guideline**	Preconception Folic acid 5 mg At booking - VTE and FGR risk assessment. Offer Vitamin D 25mg or 1000IU 16 weeks – Obstetrician ANC counselling +/- referral to dietician (IP referral on Cerner) 28 weeks +/- LMWH 32 and 36 weeks – Obstetrician ANC Discuss Postpartum prophylaxis	24-28 weeks: OGTT	4-weekly from 32 weeks till birth
BMI>40 **Raised Body Mass Index (BMI) in pregnancy guideline**	As per BMI 35-39.9 At booking – assess if equipment can adjust to patient or referral required 32 weeks for anaesthetic counselling	Appropriate cuff size 24-28 weeks - OGTT	4-weekly from 32 weeks till birth
Chicken pox **202204111056350.Chicken pox in pregnancy.pdf**	If patient develops chickenpox asks her not to attend ANC until all lesions have crusted over Confirm immunity and acute infection and treat according to guidelines Obstetric ANC after FMU scan	Bloods post exposure Ig VZV	FMU scan 5 weeks after onset of rash, signs of: - Fetal growth restriction - Limb hypoplasia - Ventriculomegaly or microcephaly - Skin or ocular lesions
Chronic Hypertension **HTN present at booking or before 20 weeks gestation** **(Antihypertensive drugs in pregnancy: 202601301510290.Antenatal Hypertension Pregnancy V5.1.pdf)**	Booking – change antihypertensives to labetalol/nifedipine/ methyldopa 12 weeks - Aspirin 150 mg 16 weeks – Obstetric ANC	DIGITAL BP should be used at all times Booking bloods including FBC and U&E – repeat 28 and 34 weeks OGTT 24-28 weeks BP and urine each visit and should be within 2- 4 weeks intervals	Anomaly + uterine artery doppler - Normal UA PI + EFW >10th – serial scan from 32 weeks - Abnormal UA PI + EFW >10th – serial scan from 28 (every 2-4 weeks) - EFW <10th at anomaly scan- FMU referral
Congenital uterine anomaly	ANC review for scans Referral to PTB clinic		Cervical length scans Renal scan Growth from 28 weeks 4 weekly till birth
Epilepsy	Preconception: 5 mg folic acid Booking –Refer to Obs Med team 16 weeks - Obs ANC – r/v seizure control and SUDEP risk factors	No drug levels needed except if frequency of seizures increase or changing doses.	All women on AED only: 20 weeks: detailed anomaly + ECHO And 4-weekly growth scans from 28 weeks (sonographer led FMU)

202405211452040.Epilepsy in pregnancy V7.pdf	All pregnant women on AED may be registered on UK Epilepsy and Pregnancy Register 28 weeks – Obs +/- Neuro 32 weeks – Obs ANC – Vit K discussion 36 weeks – Obs ANC and discuss labour and postnatal contraception		
Fetal movements **REDUCED FETAL MOVEMENTS**	**< 24 weeks gestation** • FHR sonic aid • No CTG or ultrasound routinely indicated • If never felt FM – consider FMU referral **24+0 to 27+6 weeks gestation** • FHR - sonic aid. - If fetal heart present and no risk factors: - Reassure and provide advice on monitoring movements. • **Persistent RFM-** if risk factors for fetal growth restriction or stillbirth- consider scan at first presentation **>28 weeks** • **Without risk factors:** SFH and CTG. If normal reassure. • **Risk factors for stillbirth/FGR or more than one episode.** SFH, CTG and ultrasound (if performed within 2 weeks, to consider liquor and doppler) If cCTG abnormal- senior review		Never felt movements by 24 weeks: FMU referral Persistent reduced fetal movements or RFM and a high risk of stillbirth/FGR. Growth scan within 24 hours unless a previous scan was done within 2 weeks. In this case, liquor and doppler measurements can be considered.
FGR baby: EFW/AC <3rd centile or abnormal doppler **202411210723400.FGR Guideline 2024 Hillingdon 2024.pdf**	Booking: assess risk factors for FGR (RCOG risk stratification) +/- Aspirin 150 mg IOL at 37/40 if no other concerns	Individualised care	Follow pathway +/- uterine artery dopplers 202411210723400.FGR Guideline 2024 Hillingdon 2024.pdf
Fibroid uterus (at least one of them >3cm))	Refer to Obstetric ANC when fibroids detected 32 weeks – Obs ANC + scan 36 weeks – Obs ANC + scan Discuss fibroids and risk. Plan birth according to individual care	Optimize Hb Will need x-match in labour	32 & 36 weeks
Gestational diabetes –first recognised in pregnancy **-Fbs > 5.6 and 2hr >7.8 mol/l**	Booking GDM on diet - after initial HGM (home glucose monitoring)	At Book OGTT if previous gestational diabetes as soon as possible and at 25-25 weeks if normal	If GDM diagnosed in first trimester for 4-weekly from 28 weeks till birth GDM on diet 4-weekly from 36 weeks till birth

202512191131340.gestational diabetes guideline v4.1.pdf **T1 and T2 diabetes are in a separate row**	remote review 2-3 weekly until delivery - 31-33 weeks birth DSM preparation class - 36 weeks Dr MOB and IOL discussion - IOL 40+0 – 40+6 GDM on treatment - 1-2 weekly remote r/v DSM - 36 weeks – MOB and IOL - IOL at 39-40+0	Hb1AC if diagnosis in 1st or 2nd trimester if RBS 7 or more (THH specific) OGTT (if risk factors) – at any gestation if abnormal RBS, glucosuria	GDM on treatment 4-weekly from 28 weeks till birth
Herpes **202411110820540.Genital warts in pregnancy V5.0.pdf**	Booking previous Herpes infection with history of recurrence refer to Obs ANC If present with genital warts refer to specialist midwife for sexual health	Primary infection: A. 1st/2nd Trimester: give Aciclovir po 400 mg TDS for 5d, restart at 32 weeks for suppressive --> vaginal delivery + paeds referral B. 3rd trimester/ or at least 6 weeks prior to delivery: treatment until delivery, indication: CS, paeds to attend delivery Recurrent Herpes (pre-pregnancy) Treat if active, suppression from 32 weeks, mode of birth: Vaginal unless otherwise indicated	No additional growth scans unless other indication
Hyperthyroid **TSH <0.1mlu/l or undetectable and raised T4 or T3** **Subclinical : depressed TSH with normal T3 and T4 has no adverse effect in pregnancy** **202504141031280.Thyroid guideline V3.0.pdf**	Booking refer to endocrinology	TFTS at booking TFTs every 2-4 weeks till 20 weeks >20 weeks every 4-8 weeks Every 2 weeks if dose change TRAb in first trimester: if positive or on antithyroid medication – repeat at 20 and 28 weeks FBC ion same day if sore throat, fever or rash to exclude agranulocytosis	4-weekly growth scans (including UAD) from 28 weeks for those requiring antithyroid treatment, uncontrolled Graves' disease in pregnancy or has TRAb 3X the threshold of positivity – these should be on FMU list and checked for tachycardia, goitre, FGR and hydrops
Hypothyroid **Raised TSH and low free T4 or TSH>10mIU/l** **Or TSH>2.5-10 but normal T4** **202504141031280.Thyroid guideline V3.0.pdf**	Booking: TFT 16 weeks: Obs ANC 28 weeks: Obs ANC 36 weeks: Obs ANC - If abnormal TFT adjust medication and repeat 2-4 weeks For treatment: see guidelines	Booking: Baseline TFT 16 weeks: TFT 20 weeks TFT 28 weeks: TFT TFTs 4 weeks after changing thyroxine dose No need to check TPO antibodies. If known TPO positive, and not on medication TFTS at booking and at 20 weeks. If normal no treatment or further tests.	No further Ultrasounds needed
Inflammatory Bowel Disease	Refer to Obs Med team at booking Signpost to the IBD and pregnancy website Joint care with Gastro team – medication review. Preconception - Folic acid 5 mg until 12 weeks 28 ,32 and 36 weeks review IBD any flares/ admissions	Booking bloods, urine, BP, BMI, FGR/PET risk, VTE assessment	Growth scan if on steroids or acute flare up in pregnancy - 4 weekly from 32 weeks till birth (otherwise, no scans are needed unless other risk factors)

	Individualised care and advice from Obs Med team		
IVF pregnancy	16 weeks Obstetric ANC To consider IOL from 39 weeks (currently no impact paper or guidance)		IVF alone is not an indication for scans
Large baby - LGA: AC or EFW >95th centile Or SFH >97th centile on Intergrowth chart	Routine clinic appointments 32 weeks: book scan for 36-38 weeks 36 weeks: Obstetric ANC counselling with scan review LGA for shoulder dystocia and birth trauma + MOD/ IOL	If detected at 18-21 weeks scan: OGTT or HGM if LGA detected on anomaly scan: GTT (if this is normal, consider repeating later) GTT at any stage LGA detected	SFH >97th centile - For growth scan If growth scan shows EFW/AC>/=95th centile then repeat scan at 36-38 weeks (unless >34 weeks when only if clinically indicated) If<95th centile routine care even if SFH remains raised.
Low BMI <18.5	Booking: assess risk factors for FGR Dietary advice and offer dietician referral (CERNER order)		4-weekly from 32 weeks till birth
Low PaPP-A (0.415 MoM)	16 weeks Assess booking factors for SGA Recommend aspirin if <20 weeks Add uterine artery doppler		Add uterine artery doppler and follow pathway 202411210723400.FGR Guideline 2024 Hillingdon 2024.pdf
Maternal antibodies 202502241020400.Antibodies in pregnancy V5.0.pdf	Booking 16 weeks Obs ANC If titres normal – routine F/U If rising titres or critical for FMU referral - Anti-D ≥ 4 IU/ml, - Anti-c ≥ 7.5 IU/ml, - Any anti-K detected Delivery depends on MCA doppler	Booking – G&S – if Antibodies +ve No clinically significant Ab – repeat at 28 weeks Clinically significant Ab (anti-D, anti-K, anti-c etc) • repeat antibody titre every 4 weeks until 28 weeks then 2 weekly until delivery (follow lab guidance) • Weekly G&S from 28 weeks	Routine USS as per low-risk patient unless Ab reach critical levels - Referral to FMU for MCA PSV and growth if moderate risk of HDFN (from 18 weeks) Risk Flow chart: 202502241020400.Antibodies in pregnancy V5.0.pdf
Placenta Praevia/ low lying at 20/40- Definition: >16 weeks: placenta praevia - covering Cx Os Low lying - < 20 mm from Cx Os PLACENTS PREVIA/ACCRETA/INCRETA	32 weeks – Obstetrician ANC – consider PAS referral to if previous uterine surgery and anterior placenta 36 weeks – Obstetrician referral ANC Placenta Praevia or symptomatic Low-lying placenta: Steroids at 34-35+6 weeks unless already steroid mature Placenta Praevia: Delivery via LSCS 36-37/40 if asymptomatic) and 34-36+6/40 if symptomatic placenta praevia If Low lying Placenta praevia plan timing of delivery with patient (<20mm most likely needs Caesarean Birth especially if the edge is thick) PAS referral form (send to FMU midwives' email)	From 32 weeks - weekly G&S until delivery (if no Ab)	26–28-week scan on a high-risk antenatal scanning list if: Placenta praevia (covering os) with 1 or more intermediate or 2 or more minor risk factors) OR Low lying placenta (<20mm at 20 weeks) with a 1 major/intermediate risk factor) OR Low-lying placenta (<20mm at 20 weeks) with 2 or more minor risk factors 32 weeks for placental localisation 36 weeks + placental localisation
Polyhydramnios		• OGTT (even if earlier screening normal) • Check CST	• Confirm anomaly scan (no structural abnormalities) and CST

• **Mild: AFI 25–29.9 cm or DVP 8–11 cm** • **Moderate: AFI 30–34.9 cm or DVP 12–15 cm** • **Severe: AFI ≥35 cm or DVP ≥16 cm** **202405211458230.Management of large for gestational age foetuses and macrosomia V2.0.pdf**	2-4 weekly ANC (depending on severity) Delivery is planned depending on cause and severity (no need to offer IOL for AFI <25cm. Consider IOL if AFI>25cm at 40 weeks)	• TORCH / infection screen • Rhesus status and antibodies as per routine care • Consider karyotyping / FMU referral if: ○ Structural anomalies ○ Early-onset or severe polyhydramnios	• Refer to FMU if polyhydraminios >25cm at anomaly scan • Refer to FMU if AFI>25cm and EFW/AC less than the 5th centile • Refer to FMU at any gestation if AFI>35cm • 2–4 weekly scans, increasing to a weekly scan if severe or symptomatic
Previous Caesarean Section **202506241450060.Vaginal birth after csection VBAC V7.0.pdf**	Routine care RCOG leaflet for VBAC vs CS at booking or 16 weeks if indicated Birth options clinic with Senior MW at 16 weeks if one uncomplicated Caesarean OR 16-week ANC with obstetrician if 2 or more uterine surgeries or complex Caesarean 36 weeks Obstetrician ANC Mode of Birth		No indication until other causes
Previous congenital abnormality	Depends on previous abnormality: Examples: Spina bifida 5mg folic acid Heart abnormality ECHO FMU referral Check Genetic Studies		16 week and 20-week HRAN and then according to FMU plan
Previous PET **Management for current PIH: 202601301510290.Antenatal Hypertension Pregnancy V5.1.pdf**	12 weeks – start Aspirin 150 mg Obstetrician ANC within 2 weeks of booking appointment Individualised plan	Digital BP SHOULD BE USED AT ALL TIMES	Anomaly + uterine artery doppler - Normal UA PI + EFW >10 – serial scan from 32 weeks - Abnormal UA PI + EFW >10 – serial scan from 28 (every 2-4 weeks) - Normal UA PI + EFW <10 – serial scan 26-28 weeks (individualised) - Abnormal UA PI + EFW <10 – FMU
Routine care- low risk pregnancy **202512191144050.Maternal Antenatal screening tests Guidelines v5.1.pdf**	All pregnant women 1st trimester – booking, SGA and VTE risk assessment, vaccine information 16 weeks (14-18 weeks) - Mode of birth discussion 25 weeks (Primiparous only) 28 weeks 31 weeks (Primiparous only) 34 weeks – OASI 36 weeks 38 weeks 40 weeks (Primiparous only) Offer IOL for 41 weeks	Booking bloods and Random blood sugar (RBS), MSU, BP Fetal cell free DNA if Rh –ve. Book anti-D if needed At 28 weeks: FBC and G&S, Anti-D (if Rh -ve), SFH, LMWH if VTE> 3 SFH every visit unless serial scans from 25 weeks 34 weeks: FBC offer sweep from 39 weeks	11+2 – 14+1 - dating + NT (and bloods) 18+0 - 20+6 - Anomaly + placental location (if low, follow guidelines below) Growth scan after 26 weeks if SFH: - Plots <10th centile - static growth (min 2 weeks apart) - growth not following curve SFH>95th centile: Follow LGA management
SGA baby - EFW/AC <10th centile **202411210723400.FGR Guideline 2024 Hillingdon 2024.pdf**	Booking: assess risk factors for FGR (RCOG risk stratification) +/- Aspirin 150 mg IOL at 39/40 unless any other concerns	Individualised care	Follow pathway +/- uterine artery dopplers 202411210723400.FGR Guideline 2024 Hillingdon 2024.pdf

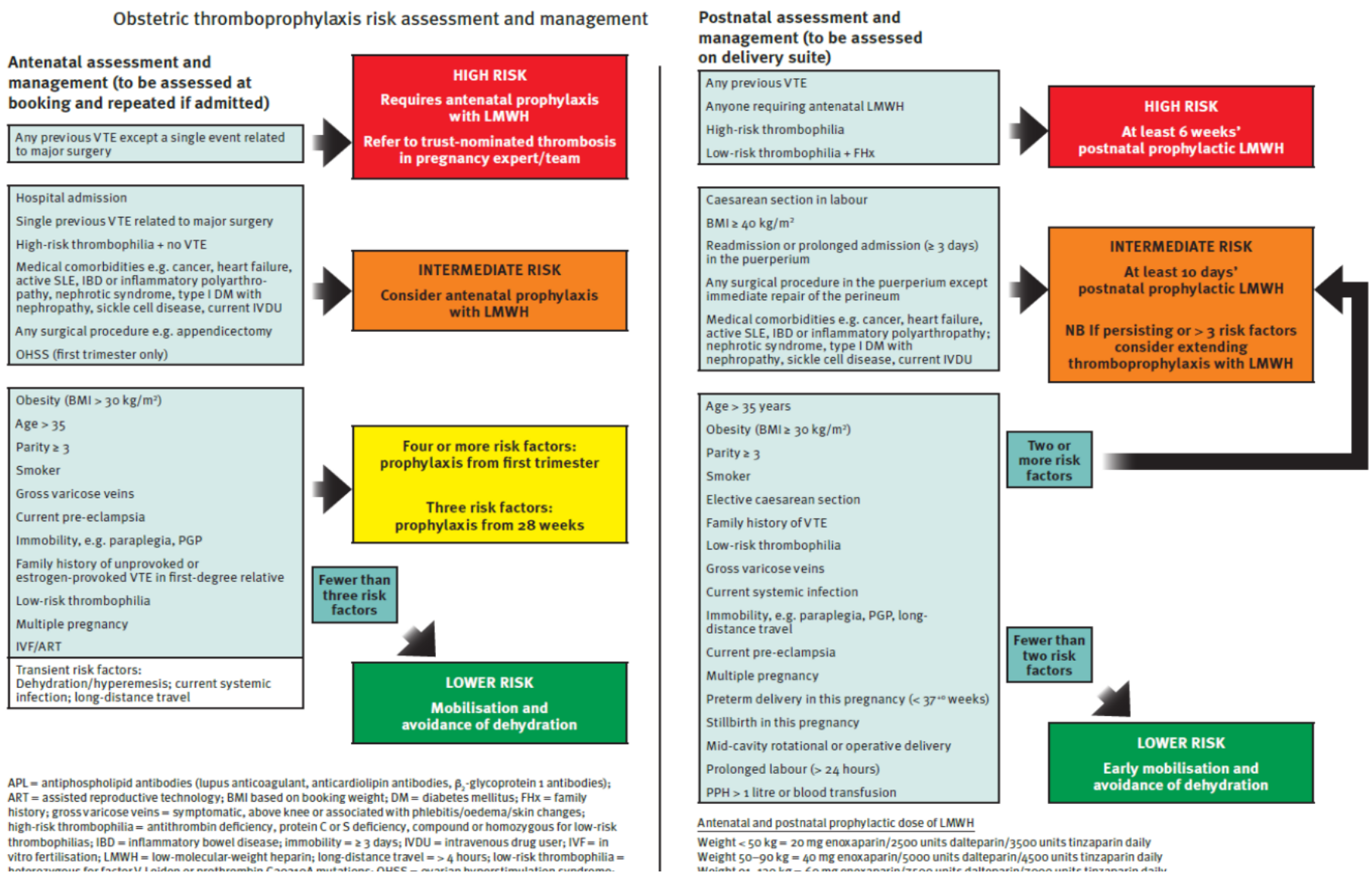
Sickle cell disease **202601231300520.Sickle Cell Disease and Pregnancy v5.0.pdf**	Preconception - Folic acid 5 mg daily, stop iron chelators, stop hydroxycarbamide (unless high risk), switch ACE/ARB to safer option - R/V vaccination – yearly influenza, if pneumococcus >5 years for repeat Booking ideally by 8-10 week + MDT review (Haematologist, Anaesthetic, Maternal medicine) Penicillin V 250 mg daily (erythromycin if penicillin allergic) From 12 weeks – start Aspirin 150 mg Monthly joint obstetric / haematology/ maternal medicine clinic At 28/40: if VTE 3, start LMWH 32/40: anaesthetic review, earlier if indicated Consider IOL at 40 weeks	BP, urine dipstick and MSU each visit Repeat FBC, renal and liver function every 4 weeks If red cell allo-antibodies detected repeat every 4 weeks Each visit assesses PET/SGA Treat anaemia ONLY if ferritin is LOW	Growth scan 4 weekly from 24 weeks till delivery
Smoker / Vaping	Counselling for smoking cessation 32 and 36 weeks Obs ANC for scan	28 weeks: one off SFH CO measurements each visit >4 ppm needs reference to NHS stop smoking service.	Growth 4-weekly from 32 weeks till delivery
Syphilis **202212011531480.Management of Positive Syphilis Serology in Pregnancy v.4.0.pdf**	Syphilis screening at booking for all pregnant people (Treponemal serology to detect IgG & IgM, e.g. EIA, CLIA or TPPA) Routine ANC appointments Referral to GUM and neonatal care to complete BASHH congenital syphilis birth plan If treatment <4 weeks prior to delivery – baby requires treatment for congenital syphilis For treatment refer to guidelines	If positive either for confirmatory test 1. send original sample for different treponemal test eg TPPA or TPHA or 2. send second sample for treponemal serology eg EIA or CLIA If confirmatory test +ve -> perform RPR or VDRL **NOTE:** notifiable disease (UKHSA) RPR or VDRL (1st prior to treatment after positive confirmatory test, then 3 monthly post treatment) Offer repeat treponemal testing (eg EIA, CLIA, TPPA) for high-risk patients 3 monthly (commercial sex worker, multiple partners, sex with MSM etc)	Consider serial growth scans for both early and late infection: if signs of congenital syphilis (eg hepatosplenomegaly, intrahepatic calcification or non-immune hydrops) for FMU referral
Thrombocytopenia **(platelets less than 150 though clinically less than a 100** **INTRODUCTION guidelines**	ITP – refer to obstetric medicine at booking	For gestational thrombocytopenia, if platelets < 80: refer to obs med. (Bloods needed include FBC, blood film and reticulocyte count, LFT, TFT, DAT, APS ab, ANA, HIV, HepB and HepC, Helicobacter pylori and consider vWF Type IIb)	No scans indicated

		If platelets <100, FBC every 2 weeks from 34 weeks or more frequently if drastic drop	
Twins – DCDA **Twin pregnancy guidelines**	16 weeks Obstetric ANC counselling, risk assessment. Aspirin if another risk factor. Folic acid 5g throughout pregnancy and consider Vitamin D Signpost to TAMBA and MBF websites Shared cared between Obstetric care and Specialist MW If cervical length <25mm, to consider progesterone pv 400mg once a day till 34 weeks) 28 weeks refer to Infant feeding and anaesthetist Mode of birth discussion by 34 weeks Aim for Delivery at 37 weeks - -	BP and urine at 20, 24 and 28 weeks then 2-weekly FBC at 24, 28 and 34 weeks	Dating scan including chorionicity, NT and aneuploidy 20 weeks – Anomaly and cervical length 4-weekly from 24 weeks till birth
Twins – MCDA **Twin pregnancy guidelines**	Obstetric ANC at 16 weeks and referral to FMU. Aspirin if another risk factor. Folic acid 5mg throughout pregnancy and consider vitamin D Signpost to TAMBA and MBF websites 28 weeks refer to infant feeding and anaesthetist If cervical length<25mm to consider progesterone pv 400mg once a day until 34 weeks Discuss mode of birth at 32 weeks Aim for delivery at -at 36/40 (36+0-36+6)	BP and urine at 20, 24 and 28 weeks then 2-weekly FBC at 24, 28 and 34 weeks	Dating scan including chorionicity, NT and aneuploidy, TTTS assessment Cervical length (single) - 16-20 weeks 16 weeks and 2weekly (check for TTS every scan) 20-22 weeks: anomaly + fetal ECHO NOTE for MCA PSV from 20 weeks (scans can alternate FMU and routine scan session if no concerns)
Two vessel Cord (single umbilical artery)	Refer for uterine artery doppler at 20-24 weeks	As SGA guidelines	Proceed to perform uterine artery doppler at 20-week scan and follow pathway No referral to FMU if found in isolation 202411210723400.FGR Guideline 2024 Hillingdon 2024.pdf
Type 1 and 2 Diabetes- prepregnant Diabetes **Type 1 and 2 Diabetes– Referral and Antenatal care** **Gestational diabetes is in a separate row**	Preconception - Folic acid 5 mg till 12-weeks Booking appt ideally by 10 weeks 11-13+6 – start Aspirin 150 mg 16 weeks 28 weeks – Anti D 32 weeks – Diabetes Clinic Obstetrician 36 weeks – Diabetes Clinic Obstetrician for discussion of Mode of Birth and IOL, colostrum harvesting, BMs ante- and postnatal check and postnatal contraception 37-38+6 weeks – IOL - Consider <37+0 if metabolic or other complications	Add Hb1AC to booking bloods Retinal assessment if retinopathy OR more than 3 months of previous retinal assessment (repeat in 3 months if positive findings), FBC, HbA1C, VTE assessment BMs – aim for fasting <5.3 mmol/L and 1hr post meal <7.8 mmol/L	20 weeks Anomaly – FMU referral if scan incomplete after 1-2 attempts 4-weekly from 28 weeks till birth – Growth + LV and doppler

Aspirin Prophylaxis

Risk level	Risk factors	Recommendation
High	• Hypertensive disease during a previous pregnancy • Chronic kidney disease • Significant autoimmune disease such as systemic lupus erythematosus or antiphospholipid syndrome (not hypothyroidism) • Type 1 or type 2 diabetes • Chronic hypertension • Low PAPP-A (<0.415 MoM) • Placental histology confirming placental dysfunction in a previous pregnancy (including placental abruption), or growth restriction/stillbirth with suspected placental dysfunction • Previous preterm birth with evidence of placental dysfunction	Recommend low dosage aspirin 150mg at night if the woman has ≥1 of these high risk factors. To commence aspirin at 12 weeks (before 16 weeks), and stop at 36 weeks gestation In some circumstances a dose of 75mg daily may be more appropriate – eg hepatic or renal disease.
Moderate	• First pregnancy • Are 40 years or older at booking • Pregnancy interval of more than 10 years • Body mass index (BMI) of 35kg/m² or more at first visit • Family history of preeclampsia in a first degree relative • Multiple pregnancy	Consider aspirin 150mg at night if the woman has two or more risk factors. In some circumstances a dose of 75mg may be more appropriate as above

VTE Prophylaxis



Appendix 4: WHO Birthweight centiles

Females:

	Centile						Centile			
Gestation	3	10	50	95		Gestation	3	10	50	95
28	915	999	1169	1383		**33+4**	1688	1853	2184	2600
28+1	932	1018	1191	1409		**33+5**	1711	1878	2214	2650
28+2	949	1037	1213	1436		**33+6**	1734	1903	2244	2671
28+3	967	1056	1236	1463		**34**	1757	1928	2273	2706
28+4	985	1075	1258	1489		**34+1**	1780	1954	2303	2742
28+5	1003	1095	1281	1516		**34+2**	1804	1979	2333	2778
28+6	1021	1115	1304	1543		**34+3**	1827	2005	2363	2814
29	1039	1135	1328	1570		**34+4**	1850	2030	2394	2850
29+1	1058	1155	1352	1598		**34+5**	1873	2056	2424	2886
29+2	1076	1176	1375	1627		**34+6**	1897	2082	2454	2922
29+3	1095	1196	1400	1655		**35**	1920	2107	2485	2958
29+4	1114	1217	1424	1684		**35+1**	1944	2133	2515	2994
29+5	1134	1238	1449	1713		**35+2**	1968	2159	2546	3031
29+6	1153	1260	1474	1743		**35+3**	1991	2185	2576	3067
30	1158	1270	1498	1783		**35+4**	2015	2211	2607	3103
30+1	1177	1292	1523	1813		**35+5**	2039	2237	2638	3140
30+2	1197	1313	1548	1843		**35+6**	2062	2263	2668	3177
30+3	1216	1335	1574	1873		**36**	2086	2289	2699	3213
30+4	1236	1357	1599	1904		**36+1**	2110	2316	2730	3250
30+5	1256	1379	1625	1935		**36+2**	2134	2342	2761	3287
30+6	1276	1401	1651	1966		**36+3**	2158	2368	2792	3323
31	1297	1423	1678	1997		**36+4**	2182	2394	2823	3360
31+1	1317	1446	1704	2029		**36+5**	2206	2420	2854	3397
31+2	1338	1468	1731	2061		**36+6**	2229	2447	2884	3434
31+3	1359	1491	1758	2093		**37**	2253	2473	2915	3471
31+4	1380	1514	1786	2126		**37+1**	2277	2499	2946	3507
31+5	1401	1538	1813	2158		**37+2**	2301	2525	2977	3544
31+6	1423	1561	1841	2191		**37+3**	2325	2551	3008	3581
32	1444	1585	1868	2224		**37+4**	2349	2578	3039	3618
32+1	1466	1608	1896	2257		**37+5**	2373	2604	3070	3654
32+2	1487	1632	1924	2291		**37+6**	2396	2630	3101	3691
32+3	1509	1656	1953	2325		**38**	2420	2656	3131	3728
32+4	1531	1680	1981	2358		**38+1**	2444	2682	3162	3764
32+5	1553	1705	2010	2393		**38+2**	2468	2708	3193	3801
32+6	1576	1729	2039	2427		**38+3**	2491	2734	3224	3837
33	1598	1754	2067	2461		**38+4**	2515	2760	3254	3874
33+1	1620	1778	2097	2496		**38+5**	2539	2786	3285	3910
33+2	1643	1803	2126	2531		**38+6**	2562	2812	3315	3947
33+3	1666	1828	2155	2565		**39**	2586	2838	3346	3983

Growth centiles

Females Cont'd:

39+1	2609	2863	3376	4019		**40+4**	2840	3116	3674	4374
39+2	2633	2889	3406	4055		**40+5**	2862	3141	3703	4408
39+3	2656	2915	3436	4091		**40+6**	2884	3165	3732	4443
39+4	2679	2940	3466	4127		**41**	2907	3190	3761	4477
39+5	2702	2965	3496	4162		**41+1**	2929	3214	3790	4512
39+6	2725	2991	3526	4198		**41+2**	2951	3239	3818	4546
40	2748	3016	3556	4233		**41+3**	2973	3263	3847	4579
40+1	2771	3041	3586	4269		**41+4**	2995	3287	3875	4613
40+2	2794	3066	3615	4304		**41+5**	3017	3310	3903	4646
40+3	2817	3091	3645	4339		**41+6**	3038	3334	3931	4680

Males:

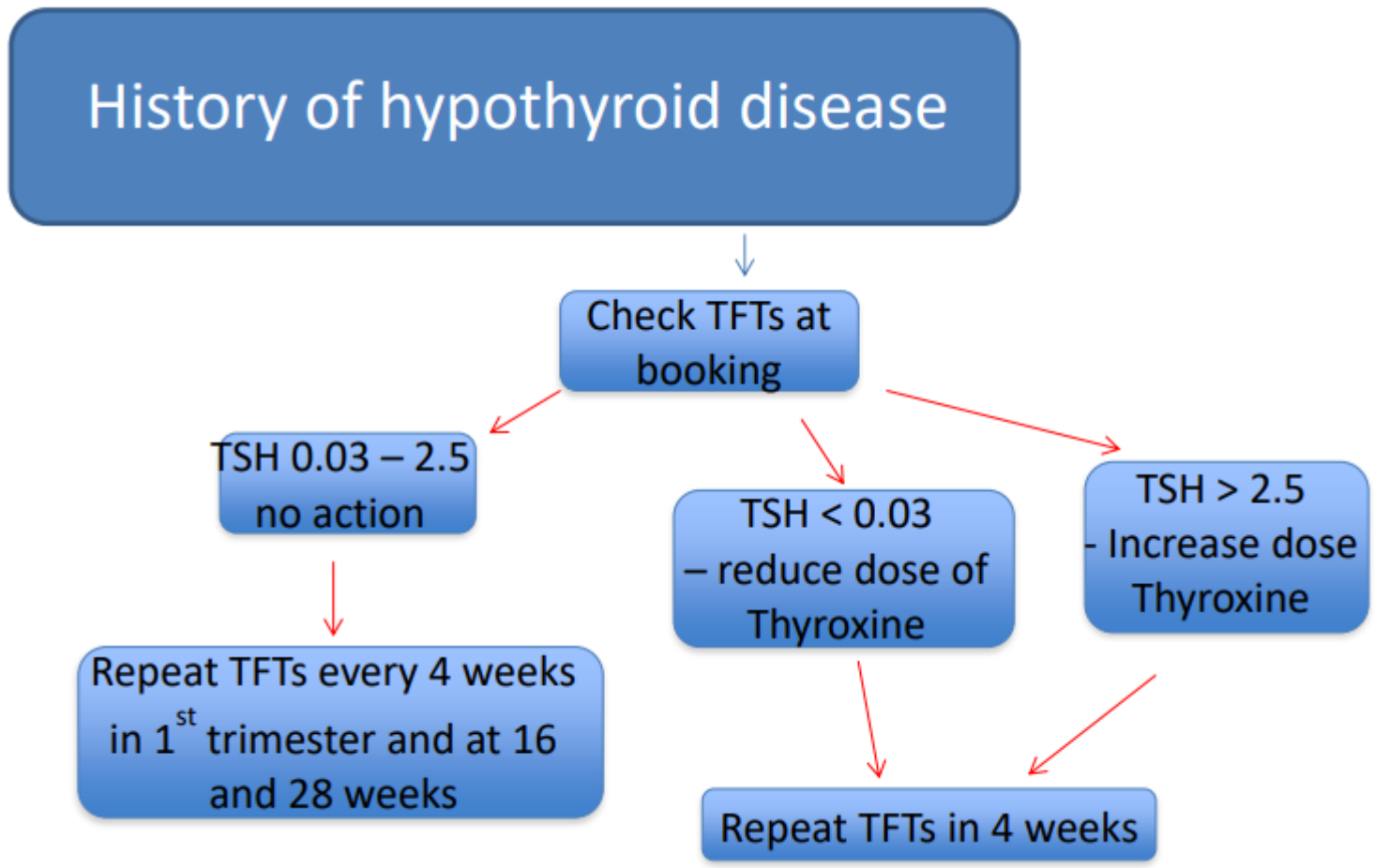
	Centile					Centile			
Gestation	3	10	50	95		Gestation	3	10	50	95
28	943	1030	1205	1424		**33+4**	1740	1909	2251	2680
28+1	960	1049	1227	1451		**33+5**	1763	1935	2281	2716
28+2	978	1068	1250	1478		**33+6**	1787	1961	2312	2752
28+3	996	1088	1273	1505		**34**	1811	1987	2343	2789
28+4	1015	1108	1297	1533		**34+1**	1835	2013	2374	2826
28+5	1033	1128	1320	1561		**34+2**	1858	2039	2404	2862
28+6	1052	1149	1344	1589		**34+3**	1882	2066	2435	2899
29	1071	1169	1368	1618		**34+4**	1906	2092	2467	2936
29+1	1090	1190	1393	1647		**34+5**	1931	2119	2498	2973
29+2	1109	1211	1417	1676		**34+6**	1955	2145	2529	3011
29+3	1129	1233	1442	1705		**35**	1979	2172	2560	3048
29+4	1148	1254	1467	1735		**35+1**	2003	2198	2592	3085
29+5	1168	1276	1493	1765		**35+2**	2028	2225	2623	3123
29+6	1188	1298	1519	1796		**35+3**	2052	2252	2655	3160
30	1193	1309	1543	1837		**35+4**	2076	2278	2686	3198
30+1	1213	1331	1569	1868		**35+5**	2101	2305	2718	3236
30+2	1233	1353	1595	1899		**35+6**	2125	2332	2750	3273
30+3	1253	1375	1621	1930		**36**	2150	2359	2781	3311
30+4	1274	1398	1648	1962		**36+1**	2174	2386	2813	3349
30+5	1294	1421	1675	1994		**36+2**	2199	2413	2845	3387
30+6	1315	1443	1702	2026		**36+3**	2223	2440	2877	3425
31	1336	1466	1729	2058		**36+4**	2248	2467	2909	3463
31+1	1358	1490	1756	2091		**36+5**	2273	2494	2940	3501
31+2	1379	1513	1784	2124		**36+6**	2297	2521	2972	3538
31+3	1400	1537	1812	2157		**37**	2322	2548	3004	3576
31+4	1422	1561	1840	2190		**37+1**	2347	2575	3036	3614
31+5	1444	1585	1868	2224		**37+2**	2371	2602	3068	3652
31+6	1466	1609	1897	2258		**37+3**	2396	2629	3100	3690
32	1488	1633	1925	2292		**37+4**	2420	2656	3131	3728
32+1	1510	1657	1954	2326		**37+5**	2445	2683	3163	3766
32+2	1533	1682	1983	2361		**37+6**	2469	2710	3195	3804
32+3	1555	1707	2012	2395		**38**	2494	2737	3227	3841
32+4	1578	1732	2041	2430		**38+1**	2518	2764	3258	3879
32+5	1601	1757	2071	2465		**38+2**	2543	2791	3290	3917
32+6	1624	1782	2101	2501		**38+3**	2567	2817	3322	3954
33	1647	1807	2130	2536		**38+4**	2592	2844	3353	3992
33+1	1670	1832	2160	2572		**38+5**	2616	2871	3385	4029
33+2	1693	1858	2190	2608		**38+6**	2640	2897	3416	4067
33+3	1716	1883	2221	2644		**39**	2664	2924	3447	4104

Males Cont'd:

39+1	2689	2950	3479	4141		**40+4**	2926	3211	3786	4507
39+2	2713	2977	3510	4178		**40+5**	2949	3236	3816	4543
39+3	2737	3003	3541	4215		**40+6**	2972	3262	3846	4578
39+4	2761	3030	3572	4252		**41**	2995	3287	3875	4614
39+5	2785	3056	3603	4289		**41+1**	3018	3312	3905	4649
39+6	2808	3082	3634	4326		**41+2**	3041	3337	3935	4684
40	2832	3108	3664	4362		**41+3**	3064	3362	3964	4719
40+1	2856	3134	3695	4399		**41+4**	3086	3387	3993	4754
40+2	2879	3160	3725	4435		**41+5**	3109	3411	4022	4788
40+3	2903	3185	3756	4471		**41+6**	3131	3436	4051	4822

Fetal growth restriction pathway

Risk Status			All Women • Routine Dating Scan between 11 and 14 weeks • Anomaly Scan 18 - 21 weeks
Low Risk	• No Risk Factors identified for FGR/ SGA • EFW at Anomaly Scan >10th centile	→	• Serial Symphysis fundal height from 26 weeks, plotted on growth chart • Units can determine locally which chart to use (customised or population) • Staff must be trained in SFH measurement, the use of a chart and referral pathways • USS referral only if indicated. See section 9

Moderate Risk Factors ***Obstetric Risk Factors:*** Previous SGA (birthweight < 10th centile** without evidence of placental dysfunction) ☐ Previous Stillbirth (birthwt appropriate for gestation) ☐***Current Risk factors:*** Smoker ☐ Drug Misuse ☐ Women ≥40 years of age at booking ☐ BMI <18.5 kg/m^2 ☐ Gastric Bypass surgery ☐ (not gastric band) Previous PTB/Second Trim misc (placenta mediated)	If EFW at Anomaly scan >10th centile → If EFW at Anomaly scan <10th centile follow High Risk pathway	• Aspirin risk assessment as below • Serial ultrasound scan from 32 weeks, every 4 weeks until delivery • Recommend delivery by 40 weeks • If undelivered at 40 weeks, then repeat scan
High Risk Factors ***Maternal Medical conditions:*** Pre-gestational diabetes ☐ Renal impairment / Chronic kidney disease ☐ Existing hypertension ☐ Autoimmune conditions, e.g. SLE, Antiphospholipid syndrome (not hypothyroidism) ☐ Major congenital heart disease ☐ Others as clinically indicated ☐ ***Obstetric history:*** Previous FGR (birthweight < 3rd centile**, placental dysfunction requiring delivery before 34 weeks, or <10th centile** with evidence of placental dysfunction) ☐ Previous SGA stillbirth Previous PET/ hypertension ☐ ***Current Pregnancy Risk factors*** PAPP-A <0.415 MoM or < 5th centile ☐ Inhibin A >2 MoMs ☐ AFP >2 MoMs ☐ Echogenic bowel ☐ EFW < 10th centile at Anomaly scan ☐ Recurrent APH [>1 significant bleed] after 13/40 ☐ Current PET ☐ Current PIH on treatment ☐ Single Umbilical Artery ☐ Severe Covid infection during pregnancy	If uterine artery Doppler available, perform at 18-23+6weeks →	*Recommend low dose aspirin as per aspirin risk assessment* ***If EFW < 10th centile at Anomaly scan:*** • Refer to FM for detailed review of fetal anatomy • Consider karyotyping, especially if <3rd centile, fetal anomalies or if uterine Dopplers normal • Serial ultrasound dependant on FM plan ***If EFW ≥10th centile and uterine artery Doppler normal:*** • Serial ultrasound from 32 weeks, every 4 weeks* until delivery ***If uterine artery Doppler abnormal (mean PI>95th centile) and EFW ≥10th centile:*** • Serial ultrasound from 28 weeks, every 4 weeks* until delivery ***If uterine artery Doppler abnormal (mean PI>95th centile) and AC or EFW <10th centile:*** • Refer to FM team (include re-review of fetal anatomy) **Consider delivery by 40 weeks unless indicated sooner** **If undelivered by 40 weeks, then repeat scan & ANC**
	If uterine artery Doppler not available →	*Recommend low dose aspirin as per aspirin risk assessment* ***If EFW < 10th centile at Anomaly scan:*** • Refer to FM for detailed review of fetal anatomy • Consider karyotyping, especially if <3rd centile, fetal anomalies or if uterine Dopplers normal • Serial ultrasound dependant on FM plan – repeat scan at latest in 3-4 weeks ***If EFW ≥10th centile at Anomaly scan:*** • Serial ultrasound from 28 weeks, every 4 weeks* until delivery **Notwithstanding the above, consider delivery by 40 weeks unless indicated sooner** **If undelivered by 40 weeks, then repeat scan & ANC**

Appendix 1 Flow chart for management of hypothyroidism